

AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT		BPA NO.	1. CONTRACT ID CODE	PAGE 1	OF 22	PAGES
2. AMENDMENT/MODIFICATION NUMBER 0001		3. EFFECTIVE DATE 05-08-2026		4. REQUISITION/PURCHASE REQ. NUMBER		
5. PROJECT NUMBER (if applicable) 460-24-009		6. ISSUED BY CODE 00244 Department of Veterans Affairs Network Contracting Office 4 Wilmington VAMC 1601 Kirkwood Highway Wilmington DE 19805		7. ADMINISTERED BY (If other than Item 6) CODE 00244 Department of Veterans Affairs Network Contracting Office 4 Wilmington VAMC 1601 Kirkwood Highway Wilmington DE 19805		
8. NAME AND ADDRESS OF CONTRACTOR (Number, street, county, State and ZIP Code) To all Offerors/Bidders		(X)		9A. AMENDMENT OF SOLICITATION NUMBER 36C24426R0087		
		☒		9B. DATED (SEE ITEM 11) 04-17-2026		
		☐		10A. MODIFICATION OF CONTRACT/ORDER NUMBER		
CODE		FACILITY CODE		10B. DATED (SEE ITEM 13)		
11. THIS ITEM ONLY APPLIES TO AMENDMENTS OF SOLICITATIONS						
☒ The above numbered solicitation is amended as set forth in Item 14. The hour and date specified for receipt of Offers ☐ is extended, ☒ is not extended. Offers must acknowledge receipt of this amendment prior to the hour and date specified in the solicitation or as amended, by one of the following methods: (a) By completing Items 8 and 15, and returning _____ copies of the amendment; (b) By acknowledging receipt of this amendment on each copy of the offer submitted; or (c) By separate letter or electronic communication which includes a reference to the solicitation and amendment numbers. FAILURE OF YOUR ACKNOWLEDGMENT TO BE RECEIVED AT THE PLACE DESIGNATED FOR THE RECEIPT OF OFFERS PRIOR TO THE HOUR AND DATE SPECIFIED MAY RESULT IN REJECTION OF YOUR OFFER. If by virtue of this amendment you desire to change an offer already submitted, such change may be made by letter or electronic communication, provided each letter or electronic communication makes reference to the solicitation and this amendment, and is received prior to the opening hour and date specified.						
12. ACCOUNTING AND APPROPRIATION DATA (If required)						
13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS, IT MODIFIES THE CONTRACT/ORDER NO. AS DESCRIBED IN ITEM 14.						
CHECK ONE	A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: (Specify authority) THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NO. IN ITEM 10A.					
☐	B. THE ABOVE NUMBERED CONTRACT/ORDER IS MODIFIED TO REFLECT THE ADMINISTRATIVE CHANGES (such as changes in paying office, appropriation date, etc.) SET FORTH IN ITEM 14, PURSUANT TO THE AUTHORITY OF FAR 43.103(b).					
☐	C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF:					
☐	D. OTHER (Specify type of modification and authority)					
E. IMPORTANT: Contractor ☐ is not, ☒ is required to sign this document and return ¹ _____ copies to the issuing office.						
14. DESCRIPTION OF AMENDMENT/MODIFICATION (Organized by UCF section headings, including solicitation/contract subject matter where feasible.) The purpose of this amendment is to: 1) Provide answers to submitted RFI's. No additional questions will be accepted. 2) Remove Fall Detection and Nurse call features / functionality from scope of work. 3) Provide ICRA requirements. 4) Add FAR clause 52.222-90 Addressing DEI Discrimination by Federal Contractors 5) Provide the site visit sign in sheet. All other terms and conditions remain the same. Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.						
15A. NAME AND TITLE OF SIGNER (Type or print)		16A. NAME AND TITLE OF CONTRACTING OFFICER (Type or print) Cheryl Roberts Contracting Officer				
15B. CONTRACTOR/OFFEROR (Signature of person authorized to sign)		15C. DATE SIGNED		16B. UNITED STATES OF AMERICA (Signature of Contracting Officer)		16C. DATE SIGNED

460-24-009 CLC and Fourth Floor Patient Elopement Upgrade

Submitted Questions & Corresponding Answers

Answers shown in red text

1. Is the VA looking to replace the existing nurse call system? That is not listed in the scope of the project. **Nurse Call System will not be replaced and is formally removed from the project.**
2. Is the VA seeking to add a fall detection system? That is not listed in the scope of the project. **Fall detection system will not be added and is formally removed from the project.**
3. If a fall detection system is included, how many fall pendants are required? **Fall detection system will not be added and is formally removed from the project.**
4. Under “adult wander management bands”, how many patient transmitters are required? **For CLC patients 60 plus 20% for overflow and charging. For the fourth floor the VA only needs 15 bands.**
5. Under “provide emergency power supplies for all equipment” please clarify, we will not be providing emergency power, we will be connecting to emergency power. **Emergency power outlets are existing in all TR and Electrical closet for use. If the contractor decides to put the headend equipment in a location that emergency power is not located, they are responsible for installation of emergency power for equipment.**
6. Please confirm whether this opportunity is restricted to Service-Disabled Veteran-Owned Small Businesses (SDVOSB), or if it is also open to other small businesses, including Minority Business Enterprises (MBEs)? **See section 10 of page 1 of the solicitation.**
7. This solicitation is classified under NAICS 238210 (Electrical Contractors and Other Wiring Installation Contractors) and issued on Standard Form 1442 (Construction, Alteration, or Repair) with Special Trade Construction limitations on subcontracting (75 percent self-performance) per VAAR 852.219-75. The scope of work, however, is the design, supply, installation, configuration, software virtualization, network integration, training, and ongoing support of an enterprise-class real-time location and patient elopement prevention system. This is functionally an electronic systems integration and IT/healthcare technology procurement, not a construction project. Federal agencies routinely procure identical systems under NAICS 541519 (Other Computer Related Services) or NAICS 561621 (Security Systems Services), which carry different size standards, bonding, Davis-Bacon applicability, and self-performance rules. Please confirm: (a) the basis for classifying this requirement under NAICS 238210 rather than 541519 or 561621; (b) whether the contracting office considered alternative NAICS codes and procurement vehicles (FAR Part 12 commercial item or FAR Part 13 simplified procedures) before issuing under SF 1442; and (c) whether the bonding (FAR

52.228-15), Davis-Bacon wage determinations, and FAR Part 36 construction clauses included in the solicitation were specifically evaluated for applicability to an electronic systems integration project of this nature. This procurement will be awarded as a construction contract based upon the heavy electrical requirements as encompassed in the SOW. Elevator cable installations, electrical distribution and emergency power installation (if need be) are significant areas of this scope and deem this a construction project. These requirements also deem the NAICS code appropriate as issued under 238210 Electrical Contractors and Other Wiring Installation Contractors. Bonding requirements, Wage Determination and clauses listed in the solicitation remain required as listed for this construction contract procurement. The questioner misreads VAAR 852.219-75, which does not require 75 percent self-performance rather it requires no more than 75 percent of the dollar value be paid to subcontractors who are not similarly situated.

8. Per VAAR 852.219-75, this solicitation is identified as Special Trade Construction with a 75 percent prime self-performance requirement (excluding cost of materials). Please confirm: (a) whether elevator travel cable installation (which under any reasonable reading falls under NAICS 238292, Other Building Equipment Contractors, and requires a licensed elevator mechanic per Davis-Bacon classification ELEV0005-003) is intended to count toward or against the prime contractor's 75 percent self-performance, or whether it is treated as cost of materials; and (b) whether the Government will recognize licensed elevator-mechanic subcontract work as outside the self-performance calculation, given that no SDVOSB elopement system integrator can self-perform licensed elevator-mechanic work without violating state elevator-licensing law. T The questioner misreads VAAR 852.219-75, which does not require 75 percent self-performance rather it requires no more than 75 percent of the dollar value be paid to subcontractors who are not similarly situated.
9. Section I.28 of the solicitation specifies general liability of \$500,000, automobile liability of \$200,000/\$500,000/\$20,000, and statutory workers' compensation. The solicitation is silent on professional liability and errors-and-omissions (E&O) insurance for the technology integration scope. Please confirm: (a) what minimum professional liability/E&O limit, if any, the VA requires of the prime; (b) whether the VA requires the prime to flow down equivalent insurance to all subcontractors; and (c) whether the VA requires certificates of insurance naming the VA as additional insured prior to subcontractors commencing on-site work. The solicitation is clear on its face.
10. Please confirm in writing that the April 28, 2026 site visit constitutes the entirety of the Government's pre-bid walkthrough opportunity, or whether a supplemental site visit will be made available — particularly given that the Contracting Officer was not present, no COR or facility engineering representative answered technical questions on the day, and several questions submitted in writing on April 20, 2026 remain unanswered. Per the requirement listed in the solicitation NLT May 5 2026 12PM EST. The pre-bid site visit as held on April 28, 2026, is the only pre-bid site visit. The questioner is mistaken when stating the contracting officer was not present.

11. Please make available, prior to the proposal due date, the AutoCAD background drawings referenced in the SOW Site Investigations section (“AutoCAD background drawings will be provided by the VA upon request”) for both Building 17 (CLC) and Building 1 (4th floor), including any existing elopement system as-built wiring diagrams. Without these, offerors cannot field-verify dimensions or scope cable runs as required by SOW Section “Site Investigations”. **Drawings were provided in the solicitation package. AutoCADs are only provided post award. Field verifications are the responsibility of the awarded contractor post award.**
12. Given that critical technical information (access control system manufacturer/model, nurse call system manufacturer/model, elevator-specific scope, door schedules, network/IT provisioning, and emergency power scope) was not available during the site visit and is the subject of these written questions, will the Government extend the proposal due date beyond May 22, 2026 by an interval at least equal to the time between the answers being published and the original due date? This is necessary for offerors to revise pricing once the answers are known. **The solicitation due date will not be extended.**
13. Please confirm whether any other prospective offerors attended the April 28, 2026 site visit, and if so how many. **The site visit attendance sign in sheet will be posted in the same amendment action as the answers to bidder questions via SAM.**
14. Per SF 1442 Block 11, the Government has specified 145 calendar days from notice to proceed for completion. Given the SOW scope (full demolition of the existing WanderGuard system across two buildings, integration with seven elevators, replacement of seven sets of elevator travel cables, emergency power installation, Class III ICRA dust containment around HVAC and patient care areas, full system commissioning, and training of up to 40 clinical personnel), please confirm: (a) whether the 145-day period is firm or negotiable; (b) whether any portion of the project may be performed concurrently with normal CLC and 4th-floor patient operations or whether some work must be performed during off-hours; and (c) whether VA-side dependencies (OIT virtual machine provisioning, biomed VLAN provisioning, elevator outages, nurse call interface windows, and access control programming) carry any committed turnaround times that the Government will incorporate into the schedule.
- a. **145 day period is firm**
 - b. **Work must be performed in a way that does not interrupt normal VA operation. If it is determined that work may disrupt VA operations during regular business hours, then the COR may require work to be performed during off hours.**
 - c. **This is incorporated into the schedule.**
15. Does the VA require real-time location visibility of every tagged patient across the entire protected floor area, displayed on an interactive floor plan accessible to patient care staff, or is the requirement limited to alarm notifications when a patient reaches a controlled egress point? The two architectures yield materially different clinical workflows: real-time floor coverage allows proactive intervention before a patient reaches an exit, whereas door-only alerting only notifies after the egress event has

begun. The VA requires real-time location visibility of every tagged patient across the entire protected floor area.

16. Were clinical/patient care staff, nursing leadership, or the Safety Officer involved in defining the functional and performance requirements in this SOW? If so, please identify their roles. This question is asked to ensure the requirements reflect the patient care team's clinical workflow needs rather than the feature set of any single vendor's product line. We will not provide the identity or the roles of who were involved during the development of the SOW.
17. When an alert is generated (elopement, fall, tamper, low battery, bed-exit, restroom call, or staff duress), must the alert uniquely identify the specific patient by name and patient identifier (MRN or equivalent), in addition to the location and alert type? Generic, location-only notifications such as "alert at Door 4" do not provide clinical staff with the information necessary to respond effectively.
 - a. Fall detection system and Nurse call will not be added and is formally removed from the project.
 - b. Follow SOW
18. Please specify the required location resolution granularity: (a) zone-level (e.g., "north corridor"); (b) room-level (e.g., "Room 412"); or (c) bed-level (e.g., "Room 412, Bed A") within multi-bed rooms. These details will be addressed post award and are not required to develop a proposal.
19. Will patient care staff (nurses, nursing assistants, security, and charge nurses) require a real-time status board or dashboard displaying all tagged patient locations on an interactive floor plan? If so, please specify (a) the number and locations of fixed workstations or wall-mounted displays needed, (b) whether mobile/tablet access is required for nursing rounds, and (c) which user groups (CLC nurses, 4th-floor nurses, security, infection control, command center) must have what level of access.
 - a. We require a headend at the main nurse station in the CLC and at each nurse stations on the 4th floor. Total of 3
 - b. Mobile/tablet access is required
 - c. CLC Nurses, 4th-floor nurses must have access, with the ability to expand to security if request by the VA.
20. Must patient tag transmissions be reliably received by the system infrastructure under realistic clinical conditions, including when the tag is (a) covered by patient clothing, hospital gowns, or bed linens; (b) located in a closed bathroom, shower, or behind a closed patient-room door; (c) in a stairwell or service corridor; or (d) within a metal-framed bed, wheelchair, or stretcher? Please specify expected detection performance under these conditions. The patient tag must be reliably received by the system under typical conditions
21. Does the system require automatic, real-time, system-wide notification when a patient tag's battery reaches a low threshold or when a tag is tampered with or removed, delivered to staff workstations and mobile devices without requiring staff to manually

scan each patient with a handheld device or tablet? Manual scanning workflows are operator-error-prone and routinely miss critical tag-status changes. **The system requires automatic, real-time, system-wide notification when a patient tag's battery reaches a low threshold or when a tag is tampered with or removed**

22. Does the VA require the system to maintain full elopement prevention functionality during a network or server outage — i.e., local controllers continue to lock doors and elevator cabs even if the LAN, server, or VLAN connection is lost? **Yes.**
23. Does the VA require automated self-diagnostics and system health monitoring that alerts both facility staff and the contractor when any component (receiver, controller, server, network segment) goes offline or degrades, without requiring manual staff checks? **Follow SOW**
24. Does the VA require documented system uptime performance standards (e.g., 99.9 percent uptime) as part of the maintenance agreement? **Follow SOW**
25. Is the system required to interface with the facility's CCTV (video surveillance) system to automatically activate cameras in the alert zone and follow a tagged patient in an alert condition, providing real-time visual tracking to security and patient care staff? **No.**
26. Must the system support temporary, authorized suspension of elopement alerts for an individual tagged patient when that patient is being escorted off the unit or off campus for a legitimate purpose (medical appointment, diagnostic imaging, family visit, scheduled outing)? Specifically: (a) can authorized staff suspend alerts for a specific patient without removing the tag; (b) does the system automatically restore alert status when the patient returns to the protected area, without requiring staff to manually re-enable monitoring; and (c) is the suspend-and-restore activity logged for audit?
- a. **Yes**
 - b. **Yes**
 - c. **Not required**
27. Does the VA wish to support staff-worn proximity override tags such that, when an authorized staff member's tag is in proximity to an at-risk patient's tag during egress through a controlled door or elevator, the system automatically recognizes the patient as escorted and suppresses the elopement alert — while logging which staff member escorted which patient through which portal at what time?
- a. **No the VA does not wish to support Staff-worn Proximity override tags in this solicitation.**
 - b. **The VA would like the functionality to expand to this system in the future**
28. Can active alerts be acknowledged and closed in person at the alerted location (door, elevator lobby, or annunciator panel) by an authorized staff member presenting a staff tag, without requiring the staff member to walk back to a central workstation? In-place acknowledgment reduces clinical response time and helps prevent alarm fatigue. **Active alerts must be acknowledged and closed in person at the alerted location.**

29. Does the VA wish to enable a programmable secondary button on staff tags for discreet, location-aware staff duress (panic) alerting, providing combined escort-authorization and duress-call capability on a single staff-worn device? **This technology is not included in this solicitation.**
30. The Wilmington VAMC has eight populated hospital floors. Does the VA anticipate, now or in the future, expansion of the same RTLS infrastructure to additional floors or buildings (a) for elopement prevention; (b) for staff duress alerting; or (c) for tracking critical mobile assets (wheelchairs, IV pumps, crash carts, ventilators, telemetry monitors)? If so, the proposed system should be architected so additional applications and additional floors can be added on the same receiver/controller/server infrastructure without replacing the elopement core.
- a. No
 - b. Not in this solicitation but potentially in the future
 - c. Not In this solicitation but potentially in the future
31. Does the VA anticipate future expansion of the elopement system to include outdoor patient tracking (courtyards, smoking areas, drop-off zones, parking areas, perimeter zones, gardens)? If so, the proposed indoor system should be architected so outdoor coverage can be added later without replacing the core infrastructure, software, or tags.
- a. Not in this solicitation but potentially in the future
 - b. We want the option to expand if necessary
32. The SOW states “New Patient Elopement technology must include integrated technologies such as fall detection and nurse call on a single platform. This platform must be portable (worn by the patient) and provide nurse staff with the location of the patient when the nurse call or the fall detection is triggered.” Please clarify whether the words “single platform” refer to a single management software platform that consolidates elopement, fall, and nurse call alerts (which the VA’s Technology Integration evaluation subfactor 1.2.1.7 appears to describe). **Fall detection system and Nurse call will not be added and is formally removed from the project.**
33. Please clarify whether VA accepts a configuration in which a single patient may wear (a) an elopement wristband and (b) a separate fall-detection device positioned for accurate trunk-acceleration sensing (clinical literature does not support reliable fall detection from wrist-worn devices), provided that both devices report through the same management platform with patient-specific identification. **Fall detection system and Nurse call will not be added and is formally removed from the project.**
34. Is fall detection required for all CLC and 4th-floor patients, or only for clinically identified high fall-risk patients? **Fall detection system and Nurse call will not be added and is formally removed from the project.**
35. What is the clinical objective of fall detection: (a) post-fall alerting (notify staff after a fall has occurred for rapid response); (b) pre-fall/bed-exit prevention (notify staff when an at-risk patient attempts to leave the bed or chair without assistance, before a fall occurs); or

(c) both? **Fall detection system and Nurse call will not be added and is formally removed from the project.**

36. Is bed-exit detection required for high fall-risk patients? Bed-exit prevention is often more clinically effective than post-fall alerting because it intervenes before the fall occurs. **Fall detection system and Nurse call will not be added and is formally removed from the project.**
37. Are patient call buttons (e.g., for use in patient bathrooms or restrooms) required as part of the elopement/patient-safety system? If so, please specify the bathroom/restroom locations requiring call buttons and the required notification path (nursing station, mobile devices, nurse call system). **Fall detection system and Nurse call will not be added and is formally removed from the project.**
38. Does the VA require any other proactive patient-safety notifications beyond elopement, fall, bed-exit, restroom call, and staff duress (e.g., patient leaving a defined safe zone, patient entering a restricted area, prolonged absence from a monitored zone)? **Fall detection system and Nurse call will not be added and is formally removed from the project.**
39. The SOW requires “patient supply of adult wander management bands (+20% for overflow and charging).” Please confirm the total patient population on the CLC and 4th floor that must be tagged, so that the +20 percent overflow can be priced accurately. **Please see RFI response #4**
40. The SOW specifies “adult wander management bands with charging.” Please confirm that any wearable form factor providing equivalent or superior elopement prevention functionality (tamper-resistant, water-resistant, continuous RF monitoring, supervised low-battery and tamper alerts) is acceptable. If a specific manufacturer/model is required, please identify the brand and model and provide the FAR 11.105 brand-name justification. **We are not requesting any specific brand in this solicitation.**
41. The site visit confirmed seven (7) elevator bays in scope. Cab nameplate identified the cab manufacturer as Elevator Controls Corporation (Sacramento, CA), with cab control cabinets bearing Magnetek HPV 900 Series 2 drives. Field information from identified the VAMC’s elevator service contractor as Allied Elevator (New Jersey). The questions #42-51 below assume those identifications and ask the Government to confirm and to clarify scope. **Noted**
42. Please confirm the seven elevators in scope and provide for each: (a) building (Building 17 CLC, Building 1, or other); (b) cab manufacturer and model (the field observation is Elevator Controls Corporation); (c) controller manufacturer, model, and series (the field observation is Magnetek HPV 900 Series 2); (d) traction or hydraulic; (e) number of floors served; and (f) age and most recent modernization date of each cab and controller. **Review SOW, all 7 are in Building 1**

43. Please confirm: (a) the VAMC's current elevator maintenance and modernization contractor (the field observation is Allied Elevator of New Jersey); (b) whether the elopement prime contractor is required to subcontract to that contractor to preserve elevator warranty and code certification; and (c) whether VA can provide written confirmation from the elevator maintenance contractor that work performed by another licensed elevator contractor will not void any active warranty or maintenance agreement.
- a. The contractor is not required to use any contractor the VA uses for project work.
 - b. The contractor is required to contact and openly exchange communication with the Wilmington VA's Elevator Maintenance contractor Elevator Technologies.
The purpose of this communication is to ensure all work is documented and will be installed correctly prior to any project work.
44. Please provide the rationale for including elevator travel cable replacement in this elopement prevention solicitation. Travel cable replacement is a major elevator capital project (NAICS 238292) that is not technically required to install elopement system tie-ins. The presence of this scope materially increases the contract value (by an estimated \$50K–\$150K depending on shaft length and cable specification, based on industry pricing for traction elevator travel cable replacement on a Magnetek HPV 900 Series 2 controller) and limits competition to elopement integrators teamed with licensed elevator contractors. Specifically, please confirm: (a) why the existing travel cables cannot be reused with three additional twisted pairs added via separate elopement-system cabling, which is the standard practice at other VA medical centers; (b) whether the existing travel cables have been inspected and found unfit for service, and if so under what code or inspection finding; and (c) whether the VA is open to removing travel cable replacement from this solicitation and procuring it separately under the appropriate elevator-modernization vehicle. It is the understanding of the Wilmington VA that the existing elevator cables are not sufficient for the elevator integration.
45. If travel cable replacement is retained in this solicitation, please provide for each of the seven cabs: (a) shaft length and number of floors served; (b) existing travel cable conductor count and gauge; (c) required new travel cable specification (conductor count, gauge, shielding, jacketing, fire rating, agency listings such as UL E-listing); (d) elevator pit and machine room access constraints; and (e) whether new travel cables must be procured through Magnetek/Allied Elevator OEM channels or whether equivalent UL-listed travel cables are acceptable.
- a. 9 floors for 4 elevators, 8 floors for 3 elevators
 - b. Follow TIL specifications for elevators
 - c. Follow TIL specifications for elevators
 - d. With prior approval and 2 week notice of COR all work can be scheduled
 - e. Follow TIL specifications for elevators
46. Was a quote for elevator travel cable replacement obtained from the VAMC's elevator maintenance contractor under the prior (cancelled) procurement? If so, please publish that quote (with supplier identity redacted if necessary) so all offerors compete on equal pricing information. The prior awardee otherwise enjoys an information advantage on the

largest variable line item in the project. The VA did not receive a quote for elevator maintenance.

47. Please confirm the Government's expectation regarding per-bay elopement integration logic. Specifically: when a tagged at-risk patient enters elevator cab N, must the system (a) lock only that specific cab from leaving the floor, with the alarm sustained until the patient exits and the cab is acknowledged by staff, while leaving the other six cabs available for normal operation; or (b) lock all seven cabs facility-wide; or (c) some other logic? e.g., functionality that supports per-cab lock-down with sustained alarm and automatic release upon patient exit and staff acknowledgment, which preserves operational continuity for staff, visitors, and other patients while the at-risk patient is contained.
- a. When a tagged patient enters a cab the system must lock only that cab from leaving the floor, while all other cabs remain in normal operation.
 - b. When a tagged patient is loitering near an elevator, the elevator must not stop at the floor.
 - c. Both conditions above must trigger an alert to both nurse stations on the 4th floor.
48. The site visit confirmed that the elevator control room is located two floors above the 8th floor (the highest populated patient floor) and is accessible only by stairs (no elevator service to the control room itself). Please confirm: (a) that VA will provide stair access for the contractor and the elevator subcontractor to the control room as needed, including for material delivery; (b) whether any material handling restrictions apply (size, weight, hours); and (c) whether VA's elevator contractor will provide labor for material handling to the control room or whether the elopement prime is responsible.
- a. The VA will provide access to rooms as needed.
 - b. The contractor must handle all material handling regardless of restrictions.
 - c. The VA will not provide guidance on how the contractor is to manage the labor of their company or their subcontractors.
49. Elopement system integration into elevator cabs typically requires (a) access to three twisted pairs in the travel cables for 24 VDC power and data tied to LF Exciters and a keypad in each cab; (b) access to the top of each cab to mount junction boxes and the LF Exciter; and (c) a licensed elevator mechanic to be present during all work in the hoistway, pit, machine room, or on top of the cab, per ASME A17.1. Will VA's elevator service contractor (Allied Elevator) provide the licensed elevator mechanic at no additional cost to the elopement prime, as is standard practice at other VAMCs? The VA's elevator maintenance contractor will not provide any services to the contractor.
50. Is "motion detection at elevators" referenced in the SOW intended to mean (a) detection of a tagged patient approaching an elevator lobby on the protected floor (an elopement-system function performed by ceiling-mounted RF readers in the lobby), or (b) physical motion detection within the elevator cab (a different function typically performed by passive infrared sensors)? These are materially different requirements with different equipment. It refers to detection of a tagged patient approaching an elevator lobby on the protected floor.

51. Please clarify whether elevator software upgrades referenced in the SOW are (a) firmware updates to the Magnetek HPV 900 Series 2 controllers (which Allied Elevator would perform under the existing maintenance agreement at no contractor cost), or (b) software changes required by the elopement system. If the latter, please describe the functional change required of the elevator controller firmware. **Software upgrades required to provide the requirements listed in the SOW**
52. The site visit identified card readers in both buildings consistent with Lenel/LenelS2 hardware (likely OnGuard head-end). The questions #53-58 below confirm and request the integration parameters. **Noted**
53. Please confirm: (a) the manufacturer, model, and software version of the existing physical access control system (the field observation is Lenel OnGuard); (b) the head-end server location and whether the elopement system must integrate at the head-end (software API) level or only at the door hardware level; and (c) the IT and security staff or vendor responsible for access control programming. **Only required at the door hardware level.**
54. Please provide the make, model, and electrical specifications (12 VDC vs. 24 VDC, current draw, fail-safe vs. fail-secure) of the maglocks or electric strikes installed at each controlled door where the elopement system must interface, including whether they are currently powered from emergency or normal power. **Refer to SOW**
55. Please confirm whether existing maglocks will be reused with the new elopement system or whether replacement is in scope. **Refer to SOW**
56. Please provide a door schedule or marked-up floor plan identifying every door requiring elopement system integration in both Building 17 (CLC) and Building 1 (4th floor), including: door type, current locking mechanism, fire-rating, and whether the door is a stairwell, exit, magnetically-held smoke door, or interior unit door. The existing RF Technologies floor plans (sheets dated 11/07/2025) show the WanderGuard exit-door geometry and label eight doors on the CLC and six to eight doors on the 4th floor, but do not indicate scope for additional doors. **Refer to SOW**
57. Is the elopement system required to integrate with the Lenel access control system at the software API level (e.g., for cardholder lookup, cardholder schedule overrides, or door-event correlation), or is dry-contact relay closure to the door locking hardware sufficient for this scope? API-level integration with Lenel OnGuard requires Lenel OpenAccess licensing or an equivalent middleware that may carry per-door or per-server licensing fees that should be specified in the solicitation if required. **Only required at the door hardware level.**

58. Are there stairwell doors in scope on either floor? If so, how many on each floor, and what is the current locking and alarming mechanism on each (magnetically-held, delayed-egress, alarm-only)? **Refer to SOW**
59. Per the SOW, nurse call integration is limited to the CLC (Building 17). Please identify the manufacturer, model, and software version of the nurse call system installed in Building 17. **Fall detection system and Nurse call will not be added and is formally removed from the project.**
60. What specific data must the elopement system pass to the nurse call system upon an alert? Specifically, must each alert message include (a) patient name and patient identifier (MRN or equivalent); (b) specific location (room, zone, door, or elevator); (c) alert type (elopement, fall, bed-exit, tamper, low battery, restroom call, staff duress); and (d) date and timestamp? Generic location-only notifications such as “alert at Door 5” do not give clinical staff actionable information. **Fall detection system and Nurse call will not be added and is formally removed from the project.**
61. Does the existing nurse call system support standard integration protocols (HL7, REST API, serial RS-232/485, TCP/IP, dry contact)? Please identify the available interface method. **Fall detection system and Nurse call will not be added and is formally removed from the project.**
62. Is nurse call integration required for all CLC nursing units, or only specific units within Building 17? **Fall detection system and Nurse call will not be added and is formally removed from the project.**
63. The SOW requires the contractor to “virtualize VHA IT-approved server enterprise software onto the Wilmington VAN facility server, communicating over the hospital’s Local Area Network (LAN).” Please confirm: (a) whether VA OIT will provide and configure the virtual machine (VM) host environment (resource allocation, OS, storage, backups, and antivirus baseline), with the contractor responsible only for installing and configuring the elopement application software within the VM; or (b) whether the contractor is responsible for VM build-out as well. **Refer to SOW**
64. The site visit identified an IT closet on the 8th floor of the relevant tower. Please confirm: (a) whether this 8th-floor IT closet is the intended location for the elopement system server (or VM host), or whether the server is intended to reside in a separate facility data center or building data closet; (b) the available rack space, power, and cooling at the intended location; and (c) any environmental or physical-security access constraints relevant to commissioning and ongoing maintenance. **Refer to SOW**

65. Is the elopement system required to operate on (a) the VA Enterprise LAN; (b) a Biomedical Engineering VLAN; or (c) a standalone, isolated network? Note for the Government's information: SecurTRAK / PatientTRAK platform holds an active VA Technical Reference Model (TRM) authorization and Enterprise Risk Assessment (ERA) approval, and currently operates on Biomed VLANs at multiple VA medical centers with multi-year zero-downtime track records. Operating on the Biomed VLAN is the technically-preferred deployment for life-safety RTLS at VA. Please confirm whether the Biomed VLAN deployment path is acceptable to Wilmington VAMC OIT and Biomedical Engineering. **The elopement system is required to operate in a way that accomplishes the goals of the SOW.**
66. How many VHA workstations require the elopement client software? Please identify the workstation locations (CLC nursing stations, 4th-floor nursing stations, security office, command center, biomed office) and the user groups using each. **Question answered refer to RFI#19**
67. Will the contractor be granted (a) remote access to the elopement server and clients via VA Citrix Access Gateway or Azure Virtual Desktop; and (b) Administrator-level privileges on the elopement system server and workstations, for ongoing configuration changes, software updates, and remote troubleshooting? **The contractor will be granted access as required to accomplish the goals of the SOW.**
68. Will VA OIT support be available to assist with network configuration, firewall rules, and remote access provisioning within a defined turnaround time? At prior VAMC installations, delays in obtaining remote access and network provisioning have extended schedules by 60–90 days, materially affecting the 145-day construction completion period in this solicitation. **Yes.**
69. Does the VA require the elopement system software to be TRM-approved prior to installation? Please confirm the version of the TRM the offeror should reference. **Refer to VA OIT Guidelines. <https://www.oit.va.gov/services/TRM/TRMHomePage.aspx>**
70. The SOW requires the contractor to “provide and install emergency power for power supplies for all patient elopement equipment.” Please clarify: (a) whether an existing emergency branch-circuit breaker slot on an existing emergency panelboard will be made available to the contractor for connection of the elopement system power supplies; or (b) whether the contractor is responsible for extending a new emergency branch circuit (or feeder) from the main emergency distribution panel — including all associated conduit, wiring, conductors, and panel-schedule updates. **If emergency power is not available in the location the contractor chooses to install equipment, the contractor must provide it.**
71. If a new emergency branch circuit is required, please identify (a) the location of the nearest available emergency panelboard relative to each elopement system equipment

location in both buildings; (b) whether VA's electrical engineering will provide arc-flash and short-circuit study updates required for any new circuit installation; and (c) the expected method of conduit run (exposed EMT, concealed in chase, or above accessible ceiling). **Drawings will be provided post award.**

72. Please confirm the expected number of separately-fused emergency circuits required: (a) one per equipment cabinet (server/VM host, central power supply, staff alert panel, controllers); (b) one per floor; or (c) one per building. **This is the contractor's responsibility.**
73. Can the existing power supplies and other infrastructure from the current elopement system be re-used to reduce costs to the VA? **Yes.**
74. The existing system is identified on the floor plans as a Stanley Healthcare WanderGuard SafePlace 9450 with a centralized hub, serial terminal servers, 16-port network switches, central power supplies, Touchpad Exit Controllers (TEC/SD keypads), Deactivation Relays (RB-8), motion detectors, proximity card readers, and 8-zone Staff Alert Wall Panels. Please confirm: (a) that this is the system to be removed; (b) whether the existing system is currently operational; and (c) whether awarded contractors standing requirement to fully commission and validate the new PatientTRAK system before any portion of the existing WanderGuard system is decommissioned will be acceptable to the COR — there will be zero gap in elopement protection at any point during transition. **This system is operational but currently failing. Zero gap in elopement protection is required.**
75. Please confirm whether any of the existing low-voltage infrastructure may be reused — specifically: (a) conduit and cable trays; (b) junction boxes; (c) network switches; (d) central power supplies; and (e) emergency power circuits feeding existing equipment. Reusing existing infrastructure could reduce the project cost to the VA. **Follow SOW**
76. Will VA be responsible for removing and disposing of the existing WanderGuard equipment, or is removal and disposal the contractor's responsibility? If contractor responsibility, please confirm whether any of the equipment must be returned to the Government (excess property, GSA disposal, etc.) or whether it may be disposed of as ordinary electronic recyclable. **Follow SOW**
77. Was the prior (cancelled) contract partially performed? If any equipment was delivered, installed, or staged on site prior to cancellation, please identify what was installed or delivered, where it currently resides, and whether it is available for offerors to incorporate. **No work had been performed; no NTP was issued on that contract before termination.**
78. The SOW requires "Class III full dust containment" around HVAC and patient care areas. Please confirm: (a) which ICRA classification (PE/IC matrix) governs each zone of work in Building 17 (CLC) and Building 1 (4th floor); (b) whether negative-air machines and

HEPA filtration are required, and whether the Government will provide or rent these or whether the contractor is responsible; and (c) whether daily entry/exit logs and signage requirements will be specified in writing or determined in the field by the COR and Infection Control. **Follow the ICRA document provided in the amendment.**

79. Please confirm whether after-hours work will be permitted and, if so, the eligible windows. Patient care floors with 24/7 occupancy typically require overnight and weekend work for any drilling, dust-generating activity, and elevator outages. SOW Section “Other Specifics” item 3 indicates after-hours work is permitted “with approval of the COR” — please confirm any standing approvals or constraints. **See SOW, work is to proceed without disruption of VA operations**
80. Please confirm whether VA will provide a defined material-staging area for both buildings, and whether secured contractor space (lock-up) will be available for tools and equipment overnight. **The VA will not provide a defined material-staging area unless approved by COR.**
81. “Repair and paint any holes or damages from installations and removals” — please confirm: (a) the painting standard required (manufacturer, color match procedure); (b) whether wall surfaces in patient rooms and corridors are subject to any hospital-specific finish standards; and (c) whether the contractor is required to coordinate paint-match samples with VA Engineering before final repair.
- a. **Contractor to coordinate with the COR.**
 - b. **Follow TIL specifications for interior finishes.**
82. The Evaluation Factors section states “Technical Capability and Past Performance are equal and when combined are significantly more important than Price” and identifies subfactors within Technical Capability of equal importance. Please confirm: (a) whether evaluation will be a best-value tradeoff or a lowest-price-technically-acceptable (LPTA) selection; (b) whether the Government intends to make award without discussions per FAR 15.306(a)(3); and (c) the relative weighting (or order of importance) of the nine subfactors enumerated under Subfactor A — Server Configuration, Staff Alert Panels, Door Management Systems, Electrical Installation, Remote Support, Patient Devices (Elopement), Patient Devices (Nurse Call/Fall Detection), Technology Integration, Commissioning, and Elevator Cable Integration. **This is a best value trade off. Relative weighting is clear in solicitation.**
83. The Past Performance subfactor specifies three to five “construction projects” similar in size, scope, and dollar value. Given that this project is functionally an electronic systems integration, please confirm: (a) whether prior VAMC patient elopement system installations performed under non-construction NAICS codes (e.g., 561621 or 541519) at other VA medical centers will be accepted as relevant past performance; and (b) whether system installations performed on multi-million-dollar VA contracts that included similar electronic integration scope will be evaluated as relevant. **Follow guidelines provided in the solicitation.**

84. Please provide the format requirements for proposal submission — specifically (a) page limits per subfactor (10 pages total for Technical Capability is stated; please confirm whether resumes, drawings, and pricing schedules are excluded from the count); (b) font, margin, and binding requirements; (c) submission method (email, SAM.gov upload, hard copy); and (d) the SF 1442 line item structure for the Schedule of Values, given that the SOW lists ten distinct cost categories (Schedule of Values items 3.2.1.1 through 3.2.1.10). **Font and margin are up to contractors' discretion. For page limitations, follow specific guidelines under Evaluation Factors Technical Capability section of solicitation. Page limitations given are specific only to the Technical Capability section. Proposals are to be emailed to the Contract Specialist at Hanna.Karmazyn@va.gov NLT May 22, 2026 at 12:00PM EST.**
85. Please confirm whether all subfactor responses to Subfactor A — System Requirements should be addressed in narrative form or whether a compliance matrix (line-item SOW response) is acceptable or preferred. **Follow guidelines provided in the solicitation.**
86. Per VAAR 852.219-75, the prime SDVOSB must self-perform 75 percent of the contract amount excluding cost of materials. Please confirm whether the following are treated as cost of materials (excluded from the 75 percent calculation) or as subcontracted labor (included): (a) elevator travel cables and cab integration hardware purchased through Allied Elevator; (b) Magnetek-OEM control software upgrades; (c) Lenel access control hardware; (d) elopement system hardware and software (CenTrak / ELPAS / SecurTRAK / PatienTRAK). **Follow definitions for what qualifies as material and what qualifies as labor.**
87. Will the Government accept teaming with a licensed elevator subcontractor for the elevator-mechanic portion of the work without that scope counting against the prime's 75 percent self-performance, given that no SDVOSB elopement integrator can self-perform licensed elevator mechanic work? **The questioner misreads VAAR 852.219-75, which does not require 75 percent self-performance rather it requires no more than 75 percent of the dollar value be paid to subcontractors who are not similarly situated.**
88. Will the Government accept teaming with a licensed Lenel-certified integrator for the access-control head-end programming portion of the work (if API integration is required) without that scope counting against the prime's 75 percent self-performance? **The questioner misreads VAAR 852.219-75, which does not require 75 percent self-performance rather it requires no more than 75 percent of the dollar value be paid to subcontractors who are not similarly situated.**
89. Please confirm whether bid bonds, performance bonds, and payment bonds are required for this acquisition, and the required amounts. The SF 1442 cover sheet (Block 12a) is marked YES for performance and payment bonds and Block 12b indicates 10 calendar days post-award. Please confirm the required bond percentages (typically 100 percent for performance and 100 percent for payment under FAR 28.102) and whether the Government will honor a Class B Surety Bond from an SBA-approved surety, or whether only Treasury-approved surety on the Department of Treasury Circular 570 list is acceptable. **Bid Bond, Performance Bond and Payment bonds are all required. Per the**

solicitation referenced clause 52.228-15 Performance and Payment Bonds- Construction, penal amount of performance and payment bonds at the time of award shall be 100 percent of the original contract price. Bonding must be through a Department of Treasury Bureau of the Fiscal Service certified company.

90. Please confirm the date by which responses to the consolidated questions in this submission will be published as a solicitation amendment. **The question is answered by this posting.**
91. Please confirm whether the Government intends to permit further written questions after the May 5, 2026 deadline if the amendment containing answers introduces new technical requirements that were not in the original solicitation. **No additional RFI's will be accepted.**
92. Per FAR 33.103(d), please confirm the agency point of contact for an Agency Protest, in the event one becomes necessary. **Protests must be served in writing to the Contracting Officer Cheryl Roberts via email at Cheryl.Roberts2@va.gov.**
93. Given the critical life-safety nature of the elopement system and the proprietary software configurations required to successfully deploy the manufacturer's infrastructure, will the Government require the bidding Prime Contractor (the SDVOSB) to provide a dated letter from the manufacturer confirming that they are formally certified on the platform being installed, configured, and commissioned? Or is the prime contractor permitted to rely solely on a subcontractor for the required manufacturer certifications and system commissioning? **Follow requirements in solicitation.**

VAMC WILMINGTON, DE
Infection Control Risk Assessment (ICRA)

Step 1. Use the following chart to identify the **Type of Construction Project Activity (Type A-D)**

Type A	Inspect and Non-invasive Activities: Includes, but not limited to: Removal of ceiling tiles for visual inspection limited to 1 tile per 50 square feet Painting (but not sanding) Wall covering, electrical trim work, minor plumbing, and activities which do not generate dust or require cutting of walls or access to ceilings other than for visual inspection
Type B	Small scale, short duration activities which create minimal dust: Includes, but not limited to: Installation of telephone and computer cabling Access to chase spaces Cutting of walls or ceiling where dust migration can be controlled
Type C	Work that generates a moderate to high level of dust or requires demolition or removal of any fixed building components or assemblies: Includes, but not limited to: Sanding of walls for painting or wall covering Removal of floor covering, ceiling tiles and casework New wall construction Minor duct work or electrical work above ceilings Major cabling activities Any activity which cannot be completed within a single work shift
Type D	Major demolition and construction projects: Includes, but not limited to: Activities that require consecutive work shifts. Requires heavy demolition or removal of a complete cabling system. New construction

Is it likely the Contractor's staff will be placed at risk for infection with tuberculosis?

YES / NO

Is it likely there will be physical disruption of the water system/lines during activity? YES

/ NO

Selected Type of Construction: D

Step 2. Using the following table, identify the Patient Risk Group that will be affected.

If more than one group is affected, select the higher risk group.

Low Risk	Medium Risk	High Risk	Highest Risk
Office areas	Cardiology Echocardiography Endoscopy Nuclear Medicine Physical Therapy Radiology/MRI Respiratory Therapy	CCU Emergency Room Laboratories Outpatient Surgery Pharmacy Post Anesthesia Care Unit Surgical Units	Any area caring for immunocompromised patients Cardiac Cath Lab Central Supply Intensive Care Units Medical Units Negative airflow rooms Oncology Operating Rooms

Select Patient Risk Group: Highest Risk

Step 3. Match the Patient Risk Group with the planned Construction Project Type (A, B, C, D) on the following matrix, to find the **Class of Precautions (I, II, III, and IV)** or level of infection control activities required.

Patient Risk Group	Type A	Type B	Type C	Type D
Low risk	I	II	II	III/IV
Medium risk	I	II	III	IV
High risk	I	II	III/IV	IV
Highest risk	II	III/IV	III/IV	IV

Select Control Procedures: III / IV

	During Construction Project	Upon Completion of Projects
Class I	1. Execute work by method to minimize raising dust from construction operations 2. Immediately replace a ceiling tile displaced for visual inspection	1. Clean work upon completion of task
Class II	1. Provide active means to prevent airborne dust from dispersing into atmosphere. 2. Water mist work surfaces to control dust while cutting 3. Seal unused doors with duct tape 4. Block off and seal air vents. 5. Place dust mat at entrance and exit of work area. 6. Remove or isolate HVAC system in areas where work is being performed.	1. Wipe work surfaces with disinfectant. 2. Contain construction waste before transport in tightly covered containers. 3. Wet mop &/or vacuum before leaving work area. 4. Remove isolation of HVAC system in area where work is being performed.
Class III	1. Remove or isolate HVAC system in area where work is being done to prevent contamination of duct system. 2. Complete all critical barriers, i.e. sheetrock, plywood, plastic, to seal area from non-work area or implement control cube method (cart with plastic covering & sealed connection to work site with HEPA vacuum for vacuuming prior to exit) before construction begins. 3. Maintain negative air pressure within worksite utilizing HEPA-equipped air filtration units. 4. Contain construction waste before transportation in tightly covered containers. 5. Cover transport receptacles or carts. Tape covering unless solid lid is used.	1. Do not remove barriers from work area until completed project is inspected by Safety Management & Infection Control & thoroughly cleaned by Environmental Management. 2. Remove barrier materials carefully to minimize spreading of dirt & debris associated with construction. 3. Vacuum work area with HEPA-filtered vacuums. 4. Wet mop with disinfectant. 5. Remove isolation of HVAC system in areas where work is being performed.

Class IV	1. Isolate HVAC system in area where work is being done to prevent contamination of duct system. 2. Complete all critical barriers, i.e., sheetrock, plywood, plastic, to seal area from non-work area or implement control cube method (cart with plastic covering & sealed connection to work site with HEPA vacuum for vacuuming prior to exit) before construction begins. 3. Maintain negative air pressure within worksite utilizing HEPA-equipped air filtration units. 4. Seal holes, pipes, conduits and punctures appropriately. 5. Construct anteroom and require all personnel to pass through this room so they can be vacuumed using a HEPA vacuum cleaner before leaving work site; or the can wear cloth or paper coveralls that are removed each time they leave the work site. 6. All personnel entering the work site are required to wear shoe covers. Shoe covers must be changed each time the worker exits the work area. 7. Do not remove barriers from work area until completed project is inspected by Safety Management & Infection Control & thoroughly cleaned by Environmental Management.	1. Remove barrier materials carefully to minimize spreading of dirt & debris associated with construction. 2. Contain construction waste before transport in tightly covered containers. 3. Cover transport receptacles or carts. Tape covering unless solid lid. 4. Vacuum work area with HEPA-filtered vacuums. 5. Wet mop with disinfectant. 6. Remove isolation of HVAC system in areas where work is being performed.
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Project #:460-24-009 Project Name: 4th Floor and CLC patient Elopement System Replacement

- **A sticky mat will be used during construction; however, it will be removed each night due to the work being located in a corridor**
- **HVAC system will be undisturbed since this project consists of simply erecting two walls.**

52.222-90 Addressing DEI Discrimination by Federal Contractors (DEVIATION APR 2026)

(a) *Definitions.* As used in this clause—

Program participation means membership or participation in, or access or admission to: training, mentoring, or leadership development programs; educational opportunities; clubs; associations; or similar opportunities that are sponsored or established by the contractor or subcontractor.

Racially discriminatory diversity, equity, and inclusion (DEI) activities means disparate treatment based on race or ethnicity in the recruitment, employment (e.g., hiring, promotions), contracting (e.g., vendor agreements), program participation, or allocation or deployment of an entity's resources.

(b) In connection with the performance of work under this contract, the Contractor agrees as follows:

- (1) The Contractor will not engage in any racially discriminatory DEI activities;
 - (2) The Contractor will furnish all information and reports, including providing access to books, records, and accounts, as required by the Contracting Officer, for purposes of ascertaining compliance with this clause;
 - (3) In the event of the Contractor's or a subcontractor's noncompliance with this clause, this contract may be canceled, terminated, or suspended in whole or in part, and the Contractor or subcontractor may be declared ineligible for further Government contracts;
 - (4) The Contractor will report any subcontractor's known or reasonably knowable conduct that may violate this clause to the Contracting Officer and take any appropriate remedial actions directed by the Contracting Officer; and
 - (5) The Contractor will inform the Contracting Officer if a subcontractor sues the Contractor and the suit puts at issue, in any way, the validity of this clause.
 - (6) The Contractor recognizes that compliance with the requirements of this clause are material to the Government's payment decisions for purposes of 31 U.S.C. 3729(b)(4).
- (c) The Contractor must include the substance of this clause, including this paragraph (c), in subcontracts at any tier, including those for commercial products and commercial services, except those where the place of delivery or performance is outside the United States.

(End of clause)

See attached document: ELOPEMENT SITE VISIT 4-28-26 SIGN IN SHEET.

End of Document